

shine® Vitamin B Complex Film Coated Tablet

Active Ingredient(s):

Each film coated tablet contains:

Vitamin B1 (Thiamine Mononitrate).....	10mg
Vitamin B2 (Riboflavin).....	15mg
Vitamin B3 (Nicotinamide).....	15mg
Vitamin B5 (Calcium Pantothenate).....	5mg
Vitamin B6 (Pyridoxine HCl).....	2mg
Vitamin B12 (Cyanocobalamin).....	2mcg

Product Description:

An orange red colour film coated tablet.

Pharmacodynamics:

Vitamin B1 (Thiamine Mononitrate)

Thiamine (as the coenzyme, thiamine pyrophosphate) is associated with carbohydrate metabolism. Thiamine pyrophosphate also acts as a co-enzyme in the direct oxidative pathway of glucose metabolism. In thiamine deficiency, pyruvic and lactic acids accumulate in the tissues. The pyruvate ion is involved in the biosynthesis of acetylcholine via its conversion to acetyl co-enzyme A through a thiamine-dependent process. In thiamine deficiency, therefore, there are effects on the central nervous system due either to the effect on acetylcholine synthesis or to the lactate and pyruvate accumulation. Deficiency of thiamine results in fatigue, anorexia, gastro-intestinal disturbances, tachycardia, irritability and neurological symptoms. Gross deficiency of thiamine (and other Vitamin B group factors) leads to the condition beri-beri.

Vitamin B2 (Riboflavin)

Riboflavin is phosphorylated to Flavin mononucleotide and Flavin adenine dinucleotide which act as co-enzymes in the respiratory chain and in oxidative phosphorylation. Riboflavin deficiency presents with ocular symptoms, as well as lesions on the lips and at angles of the mouth.

Vitamin B3 (Nicotinamide)

The biochemical functions of nicotinamide as NAD and NADP (nicotinamide adenine dinucleotide phosphate) include the degradation and synthesis of fatty acids, carbohydrates and amino acids as well as hydrogen transfer. Deficiency produces pellagra and mental neurological changes.

Vitamin B5 (Calcium Pantothenate)

Pantothenic acid is incorporated into co-enzyme A and is involved in metabolic pathways involving acetylation which includes detoxification of drug molecules and biosynthesis of cholesterol, steroid hormones, mucopolysaccharides and acetylcholine. CoA has an essential function in lipid metabolism.

Vitamin B6 (Pyridoxine HCl)

Pyridoxine, once absorbed, is rapidly converted to the co-enzymes pyridoxal phosphate and pyridoxamine phosphate which play an essential role in protein metabolism. Convulsions and hypochromic anaemia have occurred in infants deficient in pyridoxine.

Vitamin B12 (Cyanocobalamin)

Vitamin B12 is present in the body mainly as methyl-cobalamin and as adenosyl-cobalamin and hydroxocobalamin. These act as co-enzymes in the trans methylation of homocysteine to methionine; in the isomerisation of methylmalonyl co-enzyme to succinyl co-enzyme and with folate in several metabolic pathways respectively. Deficiency of Vitamin B12 interferes with haemopoiesis and produces megaloblastic anaemia.

Pharmacokinetics :

Vitamin B1 (Thiamine Mononitrate)

Thiamine is absorbed from the gastro-intestinal tract and is widely distributed to most body tissues. Amounts in excess of the body's requirements are not stored but excreted in the urine as unchanged thiamine or its metabolites.

Vitamin B2 (Riboflavin)

Riboflavin is absorbed from the gastro-intestinal tract and in the circulation is bound to plasma proteins. It is widely distributed. Little is stored and excess amounts are excreted in the urine. In the body riboflavin is converted to Flavin mononucleotide (FMN) and then to Flavin adenine dinucleotide (FAD).

Vitamin B3 (Nicotinamide)

Nicotinic acid is absorbed from the gastro-intestinal tract, is widely distributed in the body tissues and has a short half-life.

Vitamin B5 (Calcium Pantothenate)

Pantothenic acid is readily absorbed from the gastro-intestinal tract and is widely distributed in the body tissues. About 70% of pantothenic acid is excreted unchanged in the urine and about 30% in the faeces.

Vitamin B6 (Pyridoxine HCl)

Pyridoxine is absorbed from the gastro-intestinal tract and converted to the active pyridoxal phosphate which is bound to plasma proteins. It is excreted in the urine as 4-pyridoxic acid.

Vitamin B12 (Cyanocobalamin)

Cyanocobalamin is absorbed from the gastro-intestinal tract and is extensively bound to specific plasma proteins. A study with labelled Vitamin B12 showed it was quickly taken up by the intestinal mucosa and held there for 2-3 hours. Peak concentrations in the blood and tissues did not occur until 8-12 hours after dosage with maximum concentrations in the liver within 24 hours. Cobalamins are stored in the liver, excreted in the bile and undergo enterohepatic recycling. Part of a dose is excreted in the urine, most of it in the first eight hours.

Indication:

For the prophylaxis and treatment of the vitamin B complex deficiency such as glossitis, angular stomatitis, neuritis, eczema seborrhoicum, urticaria and hyperemesis gravidarum.

Dosage and Administration:

Adults: Take 1-2 tablets, once per day.

For treatment: Take 1-2 tablets, 2 times per day, depending on the age and severity of the case.

Route of Administration: Oral

Contraindications:

Contraindicated in patients who shown hypersensitivity and to any components.

Warnings and Precautions:

Problems have not been documented with intake of normal daily requirement. Studies have not been done in either animals or humans. Not intended for treatment of pernicious anaemia or other megaloblastic anaemias where vitamin B12 is deficient.

Interactions with Other Medicaments:

None has been reported.

Pregnancy and Lactation:

Considered safe in recommended dose.

Undesirable Effects:

Water-soluble vitamins seldom cause toxicity in persons with normal renal function.

Symptoms and Treatments of Overdose:

There is no specific antidote for its overdose, therefore management of the patients consist of symptomatic and supportive therapy.

Storage Conditions:

Keep container tightly closed.

Store at temperature below 30°C.

Protect from light and moisture.

Keep out of reach of children.

Jauhkan daripada capaian kanak-kanak.

Shelf-Life:

3 years from the date of manufacture.

Dosage Forms and Packaging Available:

Plastic bottle of 100's, 120's and 1,000's (For export only).

Not all pack sizes may be marketed.

Reg. No: MAL19987358XZ



Manufacturer and Product Registration Holder:

Y.S.P. INDUSTRIES (M) SDN. BHD. (199001001034)

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